18-WEEK RTT PERFORMANCE ANALYSIS: SEPT 2025-AUG 2026

October 25th 2026 @Chukwuma, U Emeka

EXECUTIVE SUMMARY

As at September 2025, 61.7% of the 7.3 million patients on the NHS England elective waiting list had waited 18 weeks or below when compared with the interim national target of 65% (NHS England) for 2025/26 reporting year and the constitutional standard of 92% (NHS England). By August 2026, the percentage of patients who had waited 18 weeks or less had increased to 65.2% out of 7.2million patients, compared with the 70% interim target for 2026/27 reporting year (NHS England) with an increase of 3.5 percentage points. There was a sharp positive change in performance observed from March 2026 that was sustained through August 2026, with 8 specialties exceeding the 70% interim mark. Most of the long waits (52-week range and above) were concentrated in a small number of specialties led by Trauma and Orthopaedics and 41.6% percentage of those waiting longer than 52 weeks were accounted for by 3 specialties (Trauma and Orthopaedic, Gynaecology, and ENT). 23 providers remained below a 60% minimum floor, while 124 providers achieved the 2025/26 interim national target of 65%. The 8 providers that met the 92% constitutional target were independent sector, specialist, community or mental health providers with a comparatively small waiting list.

This analysis used 12 months of NHS England Referral to Treatment (RTT) data, which was cleaned and validated in Python with 95% confidence intervals to compare performance among specialties, and Statistical Process Control to check for special change effects observed. The primary recommendation is that the monitoring and performance team of the NHS should target Trauma and Orthopaedics, Gynaecology, and ENT first because this group accounts for about 40% of patients waiting over 52 weeks, and are also among the 5 least performing specialties against the 18-week standard.

BACKGROUND AND ANALYTICAL QUESTION

Patients on the elective care list in England have a constitutional right to start treatment within 18 weeks of referral (NHS England), and the standard stipulates that at least 92% of the waiting list should receive care within 18 weeks (NHS England). This constitutional standard has not been met for several years, and to aim to achieve it, NHS England's 2025/26 planning guidance set an interim target of 65% by March 2026, a 5-percentage point improvement for every NHS Trust, and a 60% minimum target for all providers (NHS England). The consequence of long

waits includes delayed diagnosis and eventual treatment, which therefore worsen patients' clinical outcomes.

THE ANALYTICAL QUESTION: This analysis tries to answer the question: **HOW DID 18-WEEK PERFORMANCE CHANGE BETWEEN SEPTEMBER 2025 AND AUGUST 2026, AND WHICH SPECIALTIES AND PROVIDERS ACCOUNT FOR MOST OF THE GAP?** The analysis used the incomplete pathways (Part 2 of RTT), which involves patients still on the waiting list as at the end of each month. Performance is rated as the percentage of those patients waiting 18 weeks or below, and was calculated as the sum of the weekly bands from week 1 to week 18, divided by the total waiting list. The analysis moves from national monthly performance trends, to specialty and provider-level variations, and finally statistical evaluations.

DATASET AND DATA QUALITY

Data for the analysis was the monthly RTT full extract for September 2025 to August 2026. 12 cleaned files combined into one dataset of 615,185 rows and 113 columns, and covered 547 provider codes across the full 12 months, and 532 in August 2026. The treatment functions covered across the 12 months was 23. Data quality checks concluded the data as being fit for purpose. There were no negative counts or duplicated rows, and all the incomplete pathway row weekly bands added up exactly to the reported total rows across all months.

Most missing values were noted to be structural in nature, and not a result of errors, and each was handled on that basis:

- Blank week bands were recorded as 0 cos they meant zero patients in the band
- Total column was blank for all incomplete pathways, so total_all was used as patient count
- Two provider names (Duchy Hospital and Anglia Community Eye Service) belonged to two different facilities, so all groupings were based on provider codes, not names.
- Commissioner blanks were labelled, and not removed
- The all-specialty rows C_999 totals were removed to avoid duplicate counting

KEY FINDINGS

1. **National Performance improved across the period under analysis, but remains below target.** The share of patients waiting 18 weeks or less moved from 61.7% in September 2025 to 65.2% in August 2026. Though this exceeded the set interim target of 2025/26 of 65%, it still fell short of the target of 70% for 2026/27 by 4.8%, though has plateaued.

The total waiting list shrank from 7.3 million to 7.19 million (Figure 1: National Monthly trend.

2. **Longest waits were concentrated in few specialties.** Oral Surgery Service, ENT services, and Plastic Surgery Services had the lowest 18-week performance at 53.8%, 56.8% and 59.4% respectively. Trauma and Orthopaedics, Gynaecology and ENT accounted for 41.6% of 52-week waiters (Figure 2: performance by specialties).
3. **Very long waits are falling.** From a high of 181,947 patients in September 2025 to a low of 95,824 in March 2026, and rising to 112,958 in August 2026**.** Very long waits fell by 47% from September 2025 to March 2026, but have seen risen to 112,958, still lower than September 2025 levels by 38%.

STATISTICAL ANALYSIS

STATISTICAL PROCESS CONTROL: To check the impact of the 18-week performance noticed from February-March 2026, an SPC chart of the national 18-week performance was carried out to test whether the change was due to chance or reflected a genuine change in the process. 18-week performance was stable from September 2025 to January 2026 with a mean of 61.5%, and upper and lower control limits at 61.76% and 61.33%. There was a transition in February 2026. From March 2026 to August 2026, successive months showed a fall above the baseline upper control limit consecutively, which meets the SPC rule for special cause variation. The process change was recalculated therefore as a new phase, with a mean of 65.3% and control limits of 64.3% and 66.27%, and a step change of +3.8 percentage points. Each phase has only 5-6 points, so limits are provisional. Both sets of control limits did not overlap, so it was a sustained improvement rather than a one-off spike. One limitation of the SPC is that it only indicated a process change, but not what triggered the change, and the data does not provide reasons.

CONFIDENCE INTERVAL

To further understand the performance across providers and specialties, 95% confidence intervals were calculated for the percentage of patients waiting for 18 weeks or below, using the Wilson score method.

National Performance. National performance increased from 61.7% (95% CI 61.7-61.7%) in September 2025 to 65.2% (CI 65.1-65.2%) in August 2026. With an August waiting population of about 7.19 million, the intervals are very narrow and do not overlap, so the 3.5-point improvement is not a chance occurrence.

Worst Specialty compared with the rest. Oral Surgery was the worst performer in the 18-week performance with a performance of 53.8%, compared with 65.7% for all other specialists combined, with a difference of 11.9 percentage points (CI 11.7-12.1). The interval excludes 0, therefore the gap is not likely a chance occurrence. This practically meant that 46% of Oral Surgery patients had waited longer than 18 weeks compared to 34% for other specialties, which is about 1.4 times likelier to wait over 18 weeks.

Interpretation. The data encompasses all patients on the waiting list, and not a representative sample, and since the list is quite large, small differences are considered as statistically significant. The size of a gap and its practical importance therefore are of greater importance than significance alone.

RECOMMENDATIONS

1. For Specialties that experience greater waiting times and lower 18-week performance, increasing capacity and engaging more independent sector participation may be useful to enhance performance
2. Those providers achieving below 65% performance can be given an achievable monthly improvement trajectory, that should be reviewed at quarterly basis to gauge their performance.
3. Providers within the same specialty and region may be paired with best-performing and lowest performing levels to share pathway practice and therefore enhance regional and specialty performance.

LIMITATIONS

The above analysis only described overall waiting times performance, but does not explain the reasons behind the performance, which is beyond the scope of the analysis. The RTT data is an aggregated count and holds no patient-level details on clinical urgency or demographics, and cannot therefore show whether the long waiting times had harmful effects on the patient health outcomes. Each month is captured as a snapshot of who is waiting at that point in time. Therefore, the same patient can appear in several months and the figures therefore cannot be added across several months. The analysis did not factor performances of providers that did not submit in any given month, and these are therefore excluded.

SOURCES:

- NHS England. Consultant-led Referral to Treatment Waiting Times data, September 2025 to August 2026 full extracts.

- NHS England. 2025/26 priorities and operational planning guidance.
- NHS England. https://www.england.nhs.uk/publication/medium-term-planning-framework-delivering-change-together-2026-27-to-2028-29/
- The King's Fund (2025). https://www.kingsfund.org.uk/insight-and-analysis/blogs/nhs-priorities-2026-27-to-2028-29-system-staff-patients